First Draft

30/12/2025

Sir, when it comes to symptoms, let's keep it simple: one symptom detail in a row. Symptom - duration - 3 sliders (I think we have no option but to keep the sliders below; we should figure out something for that).

Maybe Symptom and duration side by side, with sliders below.

The "calculate start date" option should not be displayed; it should happen automatically.

There should be no compulsion to add anything; it should work with whatever minimum data the user provides.

In all places, such as symptoms, medications, and everything else, there should be a small icon to add a note.

Once they click on "Generate prescription" or "Lifechart," it shouldn't download but go to the next page within the web. For the prescription on that next page, there should be an option to print.

Coming to the life chart:

First, there will be a unified chart, only one line based on the severity average of all the symptoms. Then, on the left side, there will be options to choose.
Unified will be the default, with options to select all symptoms separately, or particular symptoms alone, or multiple.

The same applies to all charts: medications, side effects, or MSE.

This is to simplify, to make the view as simple as possible.

Afterwards, with feedback, let's decide how to make the unified one better and whether unified or separate charts are better

and when the cursor moves on the chart, only the symptom and its score at that particular time should be shown. Currently, it's showing all the details of all entered dates.

When we press on a particular point, in all charts, the data of that particular date should be displayed on that chart.

Please try to decrease the height of the charts so that all four charts appear at once on the screen, with as minimal details as possible. If needed, the user can gradually zoom in, and as they zoom in, it should become more detailed.

If the treatment is the same, a straight line appears. The charts of symptoms will be bigger, and those of others can be smaller in height by default to fit into a single screen.

And it should be customizable.

In the charts, there should be sliders on the x-axis and also on the y-axis if we zoom in a lot.

My domain is ElevenElevenhealth.in. You can use that or ElevenEleven as the logo. The color is #A5DCFE; use mostly shades of this, and others in appropriate places.
Just keep this in mind; we can decide the aesthetics later.

31/12/2025

A Simple Analogy
The final Lifechart, at a glance, should resemble an Anesthesia monitor showing ECG, SpO2, RR, and BP, as seen in an operating room or ICU.
I mean, it should display that much packed information immediately.
The user can then expand the view.

And at the end of entering details, a provision to add a Provisional Diagnosis, and below it, a Differential Diagnosis.

01/01/2026

Can we try to make the landing page like the Facebook login page used to be?
One bar at the top:
* On left, the display the project name.
* On the right, an option to log in with ID and password, and near that, an option to try without logging in.
Below that:
* On the left, taking up most of the space, a display of our lifecharts, like an ad, showing how it works by posting some complex charts, photos, and videos.
* On the right, an option to sign up with minimal details.

Modernize a bit, I have added the pic of old Facebook view in the new folder.

In Medication, Two options, one to enter brand name and generic name.
If possible, in future, let's try in a way, we enter the brand name, the generic name, will be displayed by itself.
And in the prescription part, only at medications place, it should be like the image added in a new folder named "example photos".

02/01/2026

• The score should appear immediately next to the sliders, right beside them.
• We should also improve the aesthetics of the left-side panel.
• Drug names should default to generic.
• Provide an option for doctors to set their details:
• Doctor name
• Clinic name
• KMC code
• Address
• Social media handle (optional)
• Add an option to upload and use a digital signature.
• These details should automatically appear on the generated prescription.
• At the end of the prescription, include platform branding as “Generated by Eleven-Eleven.” I mean, the prescription should be in a way, it's completely personal to doctors, completely as he's own.
• In follow-up visits, diagnosis should not be repeated by default. There should be a “Revise diagnosis” option. Clicking it should open the same options as the initial visit: provisional diagnosis and differentials. If not revised, diagnosis remains unchanged.
• In follow-up visits, symptoms, medications, side effects, and MSE should auto-fill from the previous visit. The doctor can use sliders to indicate improvement or worsening. If a symptom is absent, sliding to zero should mark it as resolved. There should be options to add new symptoms, drugs, side effects, or MSE findings. This reduces repetitive typing, since most parameters remain the same across follow-ups.
• On hover, the chart should show not only the domain but also specific details.
For example, under “Thought,” show subtypes like flight of ideas along with the score.
• Clicking on a data point on one chart should highlight the same date across all charts.
Example: clicking “sedation” on 3rd January should highlight 3rd January in all charts and display corresponding scores and details.

Second draft

1. Search option in the left, to search for the names of the patient.

2. In personal details, add an option to add the phone number of the patient. But it should be optional.

3. In the personal details, an option should be there to add the Attender Name, Relation with the patient and Reliability (yes\no)

4. In the Doctor details, also an option to add the doctor's phone number and mail ID should be present.

5. In the end of the personal detail, an option to add "Patients Personal Identification Notes", as per Doctors. It's personal for a doctor, i.e. how he remembers the patient.

6. In, the follow up, also while adding the symptoms, if it's a new symptom, or if that symptom was not present in the last visit, again the onset, progression and current status should come.

7. When they are entering details for the first time, for all, chief complaints, s\e, and MSE, there should be 3 sliders, onset, progression and current status.

8. Everywhere, when asking for duration, within the box, to the right side, an option should be there to change between hours, days, weeks, months, and years, with the default setting being the days.

9. In the Sliders, as we are now seeing the number only after clicking on it, but even on hover the mouse on it, the doctor should be able to see the number, at that particular point, so that he can just click on it once or slide accordingly.

10. While adding drugs, a drop-down menu to choose, when to take the drug, i.e. it will have 8 options like, 001, 101, 010, 111, 100, 110, 011. This on prescription will be changed into simple language, like once a day in morning or twice a day in morning and night. And in remarks or notes, they will specify, if it must be taken, before or after food.

11. Just after, Symptoms, an option to add the stressor, with a dropdown to select, with 1. Inter-personal, Familial or Relationship issues. 2. Loss, Bereavement or Separation. 3. Financial or occupational stressors. 4. Health concerns or medical illness. 5. Physical or Sexual Violence 6. Others – Specify. Specify and a option to write notes or remarks
    Next, to it an option of note to add more about it. And these stressors, on the life chart will appear as a asterisk in red, on hovering on it or clicking on it, you will get to know more about it. **Or a red dot, with its rays spreading apart, and while mentioning, just like symptoms it will have a slider, but only 5 options from 1-10, only even number, based on it the colour and the spread of the dot depends. As in the attached image, but a bit smaller than this.**

https://drive.google.com/file/d/17PHypnjMaWP5oUFHuK4LVmgFmkdMux6O/view?usp=drivesdk , this remains the same for precipitating factors too.

12. After Stressor, an option to mention about the Premorbid Personality or Personality, Like 1. Paranoid, 2. Schizoid, 3. Schizotypal, 4. Antisocial, 5. Borderline, 6. Histrionic, 7. Narcissistic, 8. Avoidant, 9. Dependent, 10. Anankastic, 11. Neuroticism (negative Emotionality), 12. Low conscientiousness, 13. Low Agreeableness, 14. High Extraversion, 15. Low Openness to experience. 16, high Introversion. And others – Specify and a option to write notes or remarks. BOTH Stressors and Premorbid Personality or Personality, just comes as a options, in the same row, side by side, on clicking on it, a new menu appears, where they can select the above mention, as a checklist, can select multiple and then save them, accordingly.

13. An option to add multiple thought and perception abnormalities. And even this for the first time should have 3 sliders.

14. In medication, an option called, “Tapering Needed”, for it a separate new interface comes to add details.

15. Criteria in the end, after mentioning the diagnosis, both ICD and DSM Of that diagnosis come there, just to see and verify.

16. Prescription, should also contain, CC, SE, MSE. Chief complaints – symptoms x durations, S/e - symptoms x durations, MSE – Thought, Perception. Only if that abnormality is there it will come, otherwise no.

17. Option to download the life chart, based on the duration mentioned.

18. And In the flow, medications, will in the last, First Symptoms (lets change the heading as Chief complaints), then MSE, then Side effects, After this, an option to “Update Life Chart”, When they do so, it will go to the life chart, there if we scroll down, the option to add medications will, come. (the same one, which was there in the previous slide will now come here), Diagnosis also comes here. And after diagnosis an option to add the “follow up date” will be present.

19. Even in the first page, after the “Update Life chart” option, an option to add the medications, diagnosis and follow up, date should be there. (If they have to add the drugs, if mainly using for prescription, before they’ll. If they are using it mainly for life chart, they’ll see the life chart, and then based on that, they’ll add drugs.

20. Based on the phone number given, and based on the follow up, date updated, we should be able to send the reminders to the patients, based on the next follow up date, once a week ago, once 3 days and once the day before through WhatsApp, It should be in a way, the message is sent by the doctor, no where our name should come.

21. We Should make the the charts completely, customizable, that is, the doctor in the side bar, decide which charts to bring on the board. As default every chart will be there on the board as discussed, but should have an option to remove a particular chart (I mean hide a particular chart), add different chart. He should be able to resize the chart, arrange accordingly, and also an an options for the doctor, to set a particular arrangement as default.

22. Even in the Charts, multiple types of representation will come, (Working on it and I'll discuss with specific experts and tell you, what will be better). Doctor should be able to change the charts and view the details at his convenience.

23. (A lot of work, should be done on how a chart looks, as that is the main, product of ours) I'll work on it and let you know, what we can do.

24. We Should Do a separate chart for each class of drugs, (Anti-psychotic, Mood stabilizers, Anti- Depressants and Sedatives). Will work on it.

13 01 2026

1. **For Anti-psychotics and Benzodiazepine, the normalization will be based on the Equivalent doses of chlorpromazine and Diazepam (It will an Option). Whereas for all, we ourselves have to create something called as Relative therapeutic percentage (relative dose intensity) i.e. RTP = (Current dose – min Therapeutic dose)/(Max Therapeutic dose – Min Therapeutic Dose). I'll create an excel, for all of these and share it to you. But everything should in backend and everything of this is only for representation on the chart. On Hovering on the Chart or Clicking on the Chart, only that particular doses that the doctor enters, will come there. (Instead of milligrams, we show the dose lies within its usual clinical range, so different medications can be compared visually), Actual doses should always be visible. (We're not comparing drugs to each other. We're showing where each drug sits within its own usual clinical range, so longitudinal strategies become visible) In Onco the same is RDI (relative dose intensity).**

2. And In the first place, will adding medications, there will be an option, called **"Taper plan"**, for every drugs, just like the notes, on clicking on it, a special dialogue box comes, where the doctor can add, the drugs, below that there will be multiples rows for, adding different doses and for different duration. And that should be appropriately represented in both life chart and the prescription. For example, during detoxification, we start with, T. lorazepam

2mg 1 1 2 for 3 days, then T. lorazepam 2mg 1 0 2 for 3 days, then T. lorazepam 2mg 0 0 2 for 3 days, then T. lorazepam 2mg 0 0 1 for 3 days, then stop. We need to provide an option to enter this kind of entries easily, and another example is SSRI, start with 10 mg and increase to 20 mg after 2 weeks, but the next follow-up will be after 1 month. And an option to send these reminder of change through WhatsApp.
For Tapering, UI reference - Effortless Medicine Tapering with TatvaPractice
For Bulk WhatsAPP Messages UI Reference -

3. **As we keep sending these kind of reminders, an option for the patient to also text back, if needed. And all those chats will only be present inside the UI after opening that patient ID and if needed the doctor can read those messages and reply to them, anything.**

4. During medication, just at the heading, and option to view equivalent doses calculator and the chart, (I'll send the details in excel soon), they can calculate there itself, that is they will have an option to enter, what drug they are using now and the doses, after that an option to enter the drug for which they need the
Patient CRM: Boost Patient Engagement | TatvaPractice equivalent dose for, and immediately after that, the dose get, calculated and displays and option comes to add that to the prescription or history.

5. Above the symptoms, we also need to add, Scales. Where the doctor gets an interface to add limited number of open source scales, (Working on knowing which are opensource and which are not), Any easy interface to mark, later the results are calculated accordingly and the Displayed, the final score is also depicted as a new chart, in the final chart. (A lot of improvements will come to this), this charts come in the view, only if they have applied any chart only. Otherwise no. And this will also become our unique part. (Lets improvise it gradually) As in MDCalc and scalesforpsych.com (I have uploaded a PDF of Scales, in that for now, lets include only CIWA-Ar (page no 83) and YBOCS (Page no 21)) . **Scales option -> Dialogue box appears -> Search bar with recommended Drop down and recommended options sorted by name.**

6. In the Medication chart, there will be shades of green and red. The background of the chart will be green if the compliance is good and will be red, if the compliance is not there. If the drug is not taken as recommended. During the follow up visit, As we keep all the drugs as it is, in the previous visit, only here in the followup visits, the medications will come, immediately after the personal information and then comes the symptoms, So that the doctor, first see, what medications is he on and then above there will be an slider, with three options, with N o compliance, then partial and then completely, and even for each drug this slider will be there, but on setting in the main slider, all the drugs individual slider, will be adjust to that and then if we need we can change the individual ones. So if the Compliance is same for all the drugs, it completely green or red or a shade in between. But if its different for each drugs, the Shades change just behind that particular drug. And clicking on it or hovering, it shows whether the compliance was present or not for that particular drug.

17/01/2026

1. Attender name, relation and reliability, can come in a single row.

2. The sliders in the chief complaints are not fully slidable, its only moving from 25% to 75%, Let all the three words be visible, Lets use “Severity at onset”, “Severity during course” and “Current severity”.

3. While selecting the Duration, instead of typing the number in one box and an select day or week in other, As in the reference video 1, when we type the number, a dropdown of days, weeks, months and years come. For example, if we type 5 days, automatically a drop down to choose between 5 days, 5 weeks , 5 months or 5 years come there. This applies to even Medications.

4. And Just above the Medications, we must give an option to “save template”. A template of those medications, and to the right above medications, and option to view the selected templates, so next time, they can select those particular group of drugs with a template and add it in one click and do minor edits if needed.

5. While entering the dose of the medication, “mg” should be the default, but just as in duration, an option to choose, “microgram” should be there.

6. The Frequency of tablet intake part, should be typable. Based on what we type the suggestions should come. Any number should be typable there. For now only button 1 is working and it is just shifting the dropdown menu and not actually getting typed.

7. All these also apply to Follow up window also.

8. In Follow up, for the symptoms which are already there, for them no need of all the three slider again, only one slider, that of “Current Severity(Status)” is enough. And that will

remain as it was in the last visit. But in follow if the doctor adds a new symptom, then all the 3 slider should come again, just as in first entry and the same is for side effects.

9. And in MSE, as you have kept the “+” icon everywhere else, just keep the same here also, a “+” icon in front of the thought, perception and affect, no need to mention “Add thought” separately. Here also no need of 3 slider for old MSE, only one of “current severity” is enough, if we add new, all the 3 sliders should come.

10. Same for the side effects, and the wy of adding duration, should be same for all.

11. And even in life chart, in the left side checklist, no need to keep a separate checklist of all unified charts together, Let there be main headings only, and there let the first checklist be of the Unified chart (Averages) and let that be checked as default.

12. Currently, we can only zoom in and zoom out of the chart, but we should be able to interact completely with the charts, as we do in trading platforms or trade view, move to side, hover at a place and zoom, so that that particular part gets zoomed in and many such basic options.

13. When we hover the mouse on the chart, its happening as expected, but when we click on the particular point, instead of the new dialogue box coming, details on all the charts at that particular date should come. (As if we are hovering on all the charts at that particular time), and if we double click on it, then, the dialogue box, which is coming now should appear.

14. As after entering the details and Generate Life chart buttons, below it there is diagnosis, medications, follow date and notes in the first page. The same should also be there in the second page after the display of all the life charts and scrolling down. (The idea is, only after looking at the life chart, the doctor, confirms the diagnosis, add medications based on the insights given by the lifechart, and then add the follow up date) (The reason for also retaining it in the first page, is for doctors who only use this product for prescription generation and date and not use lifechart.

15. The followup date will be automatically set to the same date of the next month.

Our competitors (In general in medicine) for Reference of UI

1. **Healthplix**
2. **EkaCareHQ**
3. **TatvaPractice.in**

20-01-2026 (For Designing as discussed with Shashank)

1. Login screen
2. Doctors Dashboard
3. Scales dashboard
4. Symptoms template - Preformed templates will be available for all major diseases and it will be completely customisable.
5. Medications template - Customizable templates.
6. Lifecharts- completely customizable, resizable, DASHBOARD.
7. Beside Medications template - a option of “Equivalent dose calculator”.
8. After each drug, an option “taper plan”

- Colour Code - #a5dcfe

- **FONT - Arial**

DASHBOARD

1. **Add New Patient**
2. **Search Patients**
3. **Today's appointments (Confirmed (default)Pending (collapsed))**
4. **Next Appointment. (Name and Last Visit and Timing)**
5. **Doctor Profile**
6. **Notifications**
7. **Calender (On selecting the date, the patient's for that day, shoes in the dashboard)**
8.

17/02/2026

Signup Page

1. **Forgot Password and Remember me are not working. (less priority, can be done later)**

New Patient

1. **Address option and ID Notes is resizable – no need of it.**

2. **Change ID to Identification Notes.**

3. **In relation option – a dropdown Menu should come with options of Mother, Father, Sister, Brother, Uncle, Aunt, Friend, Brother In Law, Sister In Law, father in Law, Mother in Law.**
   they can also type, while typing this options, these should come as suggestions. And they can also type others.

4. **While Selecting Stressors, the alignment is not proper, rectify it and Done Button, should be in a highlighted colour and clicking anywhere outside the dialogue box, the dialogue box should disappear. If they Choose Others, a small option to Specify Stressor, should appear and the same should be depicted in the life chart with an asterisks. (I'll Also ask Shashank to work on the design for the same)**

   And these stressors, on the life chart will appear as a asterisk in red, on hovering on it or clicking on it, you will get to know more about it. Or a red dot, with its rays spreading apart, and while mentioning, just like symptoms it will have a slider, but only 5 options from 1-10, only even number, based on it the colour and the spread of the dot depends. As in the attached image, but a bit smaller than this. https://drive.google.com/file/d/17PHypnjMaWP5oUFHuK4LVmgFmkdMux6O/view?usp=drivesdk , this remains the same for precipitating factors too. Right next to the stressor there will be sliders.

5. **Exact Same changes as Stressors should be done to Premorbid Personality also.**

6. **Chief Complaints – Above that an option of templates. A list of preformed Templates will be available. I'll send the list with the name. they can also create new template or modify the existing one.**

   For Now, Do One, Template Title Schizophrenia, template contents – 1. Suspiciousness, 2. Fearfulness, 3. Talking to Self, 4. Verbally Abusive, 5. Sleep Disturbances.

7. **In medication, an option called, "Tapering Needed", for it a separate new interface comes to add details. Where the same drug name appears, multiple times, and there is an option to change the dose, frequency and so on. And the system should**

understand that the first row details is for the initial days and only after that the second row and so on and the same should be translated in the prescription.(And In the first place, will adding medications, there will be an option, called "Taper plan", for every drugs, just like the notes, on clicking on it, a special dialogue box comes, where the doctor can add, the drugs, below that there will be multiples rows for, adding different doses and for different duration. And that should be appropriately represented in both life chart and the prescription. For example, during detoxification, we start with, T. lorazepam 2mg 1 1 2 for 3 days, then T. lorazepam 2mg 1 0 2 for 3 days, then T. lorazepam 2mg 0 0 2 for 3 days, then T. lorazepam 2mg 0 0 1 for 3 days, then stop. We need to provide an option to enter this kind of entries easily, and another example is SSRI, start with 10 mg and increase to 20 mg after 2 weeks, but the next follow-up will be after 1 month. And an option to send these reminder of change through WhatsApp.
For Tapering, UI reference - Effortless Medicine Tapering with TatvaPractice)

8. Based on the phone number given, and based on the follow up, date updated, we should be able to send the reminders to the patients, based on the next follow up date, once a week ago, once 3 days and once the day before through WhatsApp, It should be in a way, the message is sent by the doctor, through the number provided by the doctor in the doctor details, no where our name should come.

9. During medication, just at the heading, and option to view equivalent doses calculator and the chart, (I'll send the details in excel soon), they can calculate there itself, that is they will have an option to enter, what drug they are using now and the doses, after that an option to enter the drug for which they need the Patient CRM: Boost Patient Engagement | TatvaPractice equivalent dose for, and immediately after that, the dose get, calculated and displays and option comes to add that to the prescription or history.

10. Above the symptoms, we also need to add, Scales. Where the doctor gets an interface to add limited number of open source scales, (Working on knowing which are opensource and which are not), Any easy interface to mark, later the results are calculated accordingly and the Displayed, the final score is also depicted as a new chart, in the final chart. (A lot of improvements will come to this), this charts come in the view, only if they have applied any chart only. Otherwise no. And this will also become our unique part. (Lets improvise it gradually) As in MDCalc and scalesforpsych.com (I have uploaded a PDF of Scales, in that for now, lets include only CIWA-Ar (page no 83) and YBOCS (Page no 21)) . Scales option -> Dialogue box appears -> Search bar with recommended Drop down and recommended options sorted by name.

11. The Frequency of the Medication, should be in a way, only one number can be typed in one box, and after typing one number it jumps to other number. And If they

Type the Letter “H” it becomes ½, in the box, and the same gets translated in the Prescription.

12. After MSE, Add a section with the Title - “Safety and Medical Profile”.
    In that, three sections,
    1. Drug Allergies/ hypersensitivities
    2. Medical Comorbidities
    3. Current Non-Psychiatric Medication
    None of these are Mandatory,
    But if entered, All of these, should be mentioned in the prescription.

13. Instead of New patient & First Visit, replace with “New Registration”. And below that like other headings, Mention “Socio-Demographic details” as the heading for first section.

Life Chart

1. The Life chart should be completely movable. i.e If there is empty space, the user should be able to drag the chart and place it in a desired place. (To help compare between Charts)

2. While going back from life Charts, There should be an option to go back to patent details also, also with the Dashboard. (Now there is only one option to go back to dashboard)

3. Rename the “Chief complaint” chart as “Symptom trajectory” chart. Stills the chief complaints entered in the patient details will only appear here.

4. In medications chart, there is a point on each day, reduce the size of that circle, and the medication chart is not displayed appropriately, while trying to download it.

Dashboard

DASHBOARD

1. Upper bar - Search patients | My Profile | Notifications
2. Right sidebar - Calendar, defaulting to the current date. Below that, two options: Change Appointment Timings and Add New Appointment, followed by Add Note. Below that, a display of today's notes.
   a. Change Appointment Timings - Default slot timings will be 9 AM-5 PM and 5 PM-10 PM, with slot timings of 30 minutes for new patients and 15 minutes for follow-up cases. If changes are needed for a particular day, they can be made here.
b. Add New Appointment, then set slot timings and duration options.

3. In the main large screen - Four options in boxes at the top: New Patient Registration | Today's APPOINTMENTS (Schedule) (with the number of scheduled and remaining appointments displayed in small text) | My Patients (total number number al patients displayed) | Scales.
4. Below that is a display area, which will default to Today's Appointments. If "My Patients" is clicked, that information will be displayed there. If the date in the calendar is changed, for that date appointments will be dispatched there.
a. Default - Today's appointment list with timings, with free slots left blank.

b. Today's Appointment list - Name | Age/Sex | Follow-up/New Case | Status (color-coded) as confirmed, pending, postponed | View Details.
   c. My Patients list - ID | Name | Age/Sex | Diagnosis | Identification Points (Note) | View Lifechart | Add Follow-up.

Changes (22/02/2026)

Sir,
First, on the left top will be our logo. On the right edge will be the search bar, followed by "My Patients" and "Notifications" (represented by a bell icon).

In the right box, there will always be a full, classic calendar. The user can directly change the dates on it. Below the calendar, in a small text size, will be the current appointment timings for that day. The rest is the as it is.

Let time be in a range, e.g., 9-9:45 am.
Let the available slot be a slightly different color.
Let "postpone" not be red.

In the top left corner of the top bar, there will be three lines, like in ChatGPT or Gemini. Clicking on that will make a sidebar appear from the left. (This will be present in all tabs.) The top bar will have three icons (Hamburger menu).

Contents of the sidebar:
1. Small search bar
2. Dashboard (Home)
3. My Patients
4. Appointments
5. Scales
At the bottom:
My Profile, Settings, Help, and Logout.

Even in the patient details entry form, the sidebar should be a hamburger menu.

27/02/2026

DASHBOARD

1. Remove the My Patients options from the Top bar.
2. Add a button in the top bar, to change between dark and light mood.
3. Reduce calendar size, After 3 options, Today's Notes should be displayed.
4. In today's Appointments. Doctor Identification details, Diagnosis and the Type of Follow up should be visible.
5. On the Top bar
    a. Increase the height of the Top bar by a bit.
    b. after the Hamburger menu, Symbol of our product with the name after it, as it is in the LOGO (Continuum big and psychiatry smaller)
    c. In Centre, The name of the Clinic, Which the doctor gives in the doctor details will be displayed.

05/03/2026

Suggested Platform Improvements:

1. **Terminology Update:** Replace "empowering clinical intuition" with the more precise "empowering clinical acumen." ✔
2. **Consistent Navigation:** To facilitate easy navigation across the entire platform, ensure the top bar and the hamburger menu bar are consistently visible at all pages (even the lifechart), not just limited to the dashboard.

08/03/2026 ✔

1. After the calendar, Change the name “Appointment Timings” to “Clinic hours”. Make the Timings as Clickable Badges, and place them to the right of the Clinic Hours text, one below the other. On clicking on the badges, only that (Morning or evening) details should be visible on the Appointments dashboard.

    a. Clinic Hours ✏️ [🌞 9:00–5:00 PM]

    [🌙 5:00–10:00 PM]

    b. On clicking on the ✏️(edit option), the user should be able to edit the clinic hours.

2. Remove the change hours, Add new appointment below the calendar. Place a “+” icon on the Top bar to the right side of the light & dark button and give three options there.

    a. Note/ Reminder

    b. New patient Registration

    c. Schedule Appointment

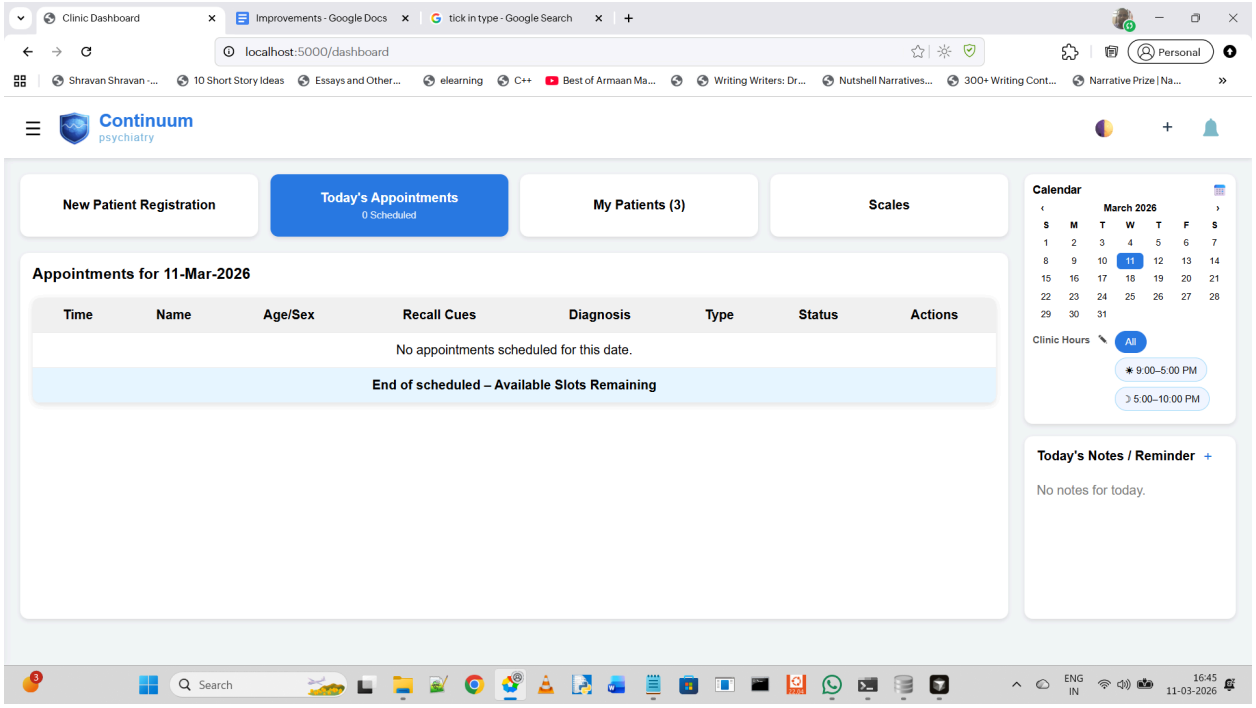
    d. Edit Clinic Hours

3. In the Same way, next to Todays note, add a “+” icon, to add a note. There they should able to change the date at which the user needs to add note at.

    a. Change Today Notes into **“Today’s Notes/ Reminder”**

    b. On clicking on it, small quick-add popover, with today’s or the date selected in calendar as default, on clicking on it, user can change the dates. And below that only in the pop up, the user can enter the note.

4. In today’s Appointments. Patient Recall Cues and Diagnosis should be visible.



21/03/2026

Proposed Feedback System

The following outlines a set for implementing a robust user feedback system:

I. User Interface Implementation

1. **Feedback Button Location:** A "Feedback" button (e.g., (🚩 Feedback)) should be placed on the top bar, positioned to the extreme right, following the notification icons.
2. **Display:** Clicking the button should open a small, self-contained box directly within the current view.

II. Feedback Submission Form Details

The form should collect the following information:

1. **Issue Type Selection:** Users must select the most relevant category:
    - Something not working (Bug)
    - Feature Suggestion
    - UI / Usability Issue
    - Performance Issue (Slow/Lag)
    - Data Issue
    - Others (requires specification)
2. **Attachments:**
    - **Screenshot:** The option to attach a screenshot of the current page should be **ON by default**. A single-click toggle allows users to opt out.
        - *Process:* Clicking the screenshot option should request permission, capture the screen, open an annotation tool for marking up the image, and then automatically attach the saved image to the feedback submission.
    - **File Upload:** Option to upload an additional image or file.
3. **Priority Setting:** Users select one of the following priority levels:
    - Minor
    - Important
    - Urgent

III. Automatic Data Capture (for Debugging)

The system must automatically capture and attach the following essential data points for debugging purposes:

- Page Name
- Timestamp
- User ID
- Other necessary technical details (to be determined by the development team).

IV. Data Storage and Access

1. **Storage:** All submitted feedback and captured data must be stored in a central database.
2. **Internal Interface (Teach Team Access):** A dedicated interface (dashboard) is required for the internal team to access and manage feedback. This dashboard should display:
    - Username
    - Page Name
    - Feedback Type
    - Message
    - Attachments
    - Timestamps
    - Status (Open/Resolved)

V. Initial Notification

Initially, upon submission, an email containing the feedback details should be sent to the company's designated email address. (The dedicated tech dashboard can be developed and gradually implemented after this initial step.)

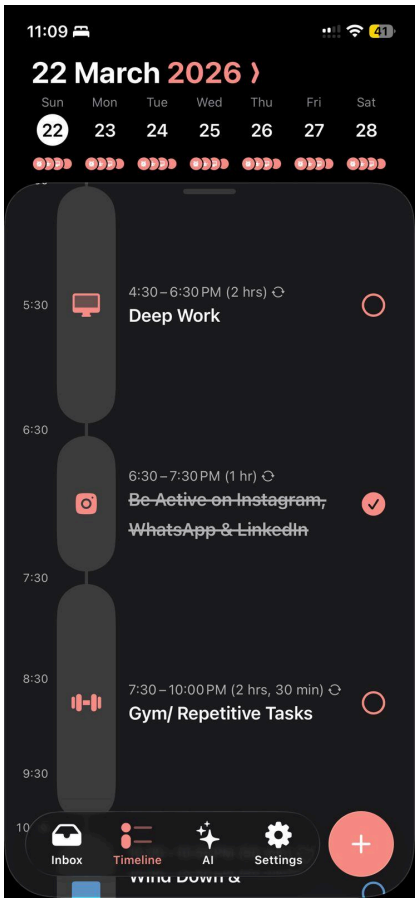
The top bar elements should be arranged from right to left as follows:

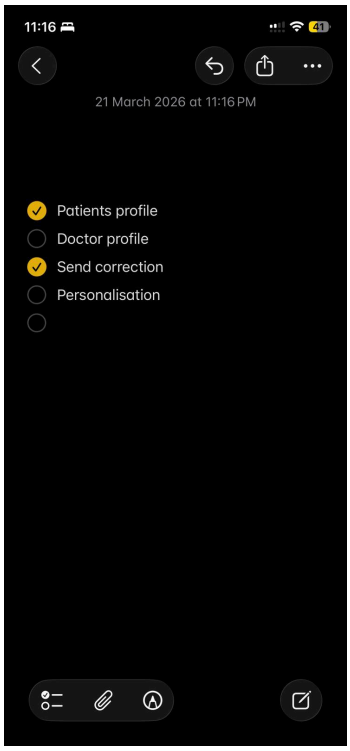
1. Profile (including name and photo, based on the Figma design)
2. Light/Dark mode toggle
3. Notifications
4. Feedback
5. Add (plus button)
6. Search
7. Clinic Name
8. Our LOGO
9. Hamburger Menu button (Extreme left)

Suggested Improvements for the Notes Feature (as a To-Do List)

The Notes feature should function as a checklist-style To-Do list, incorporating the following functionalities:

1. **Visibility:** Tasks should be presented as a clear checklist.
2. **Completion Logic:**
    - When a user (e.g., a doctor) marks a task as complete (ticks the box), a strike-through line should appear over the text.
    - The completed item should become visually less prominent (e.g., reduced transparency).
    - The completed task should automatically be moved to the bottom of the list, allowing uncompleted, higher-priority tasks to be more visible at the top.
3. **Standard Checklist Functionality:** The checklist should operate with standard, expected behaviors.


11:09
22 March 2026
Sun Mon Tue Wed Thu Fri Sat
22 23 24 25 26 27 28
5:30
4:30 – 6:30 PM (2 hrs)
Deep Work
6:30
6:30 – 7:30 PM (1 hr)
Be Active on Instagram, WhatsApp & LinkedIn
7:30
8:30
7:30 – 10:00 PM (2 hrs, 30 min)
Gym/ Repetitive Tasks
9:30
Inbox
Timeline
AI
Settings


11:16
21 March 2026 at 11:16 PM
Patients profile
Doctor profile
Send correction
Personalisation

Doctor Details

1. To Set appointment timings for particular days, with slot timings.
2. Set - Free time for Walking in patients.

27 02 2026 ✔

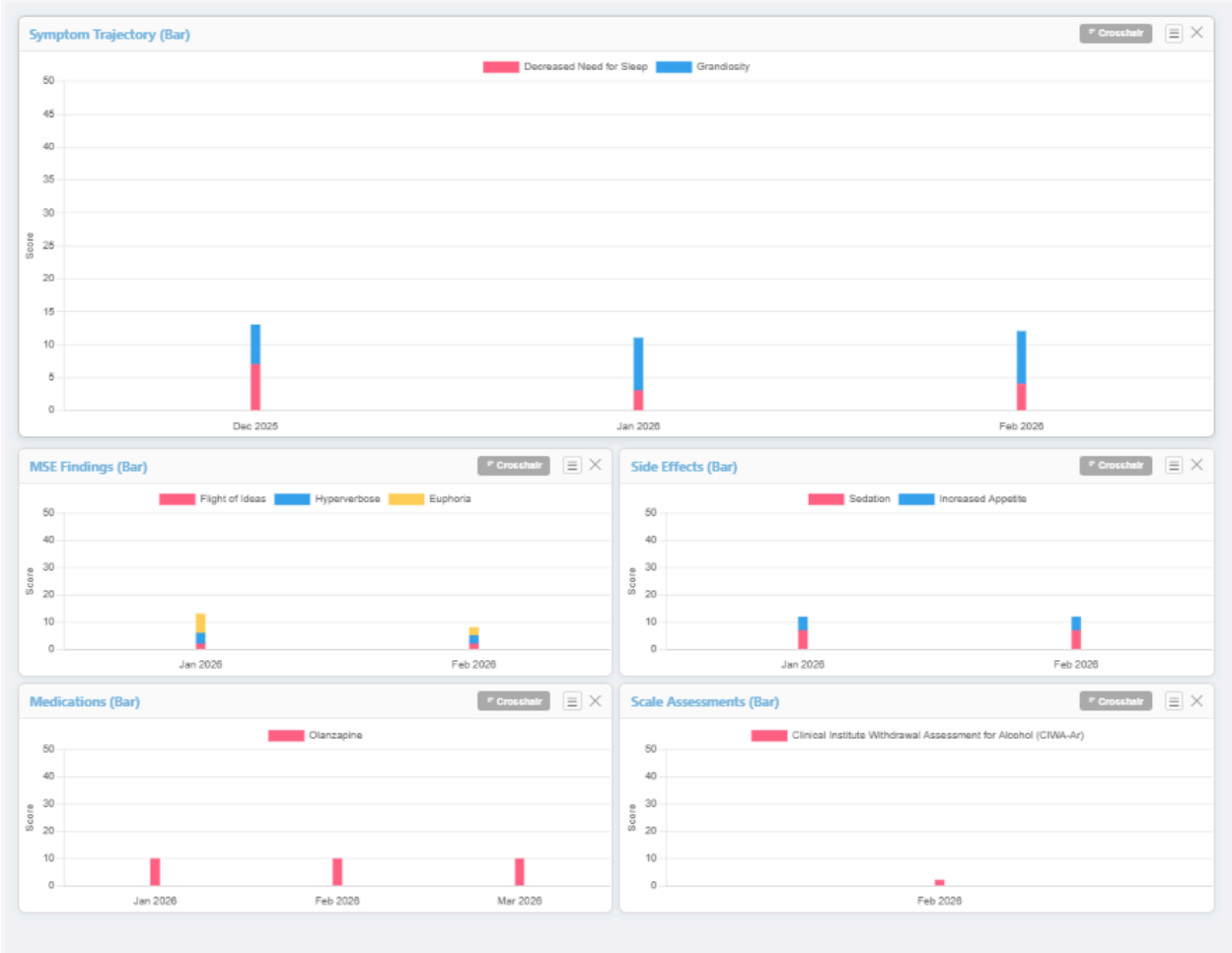
1. An option to set Usual Appointment Timings. (Default timings)
2. An option to set Default Slot Duration timings
    a. 3 types of slots and can set timings accordingly
        i. New Registration
        ii. Extended Follow up
        iii. Follow up
        iv. Therapy Session
        v. Review/ Report Discussion
    b. Doctors can add new template, Rename, and mainly Adjust Default timings and can change for individual dates.

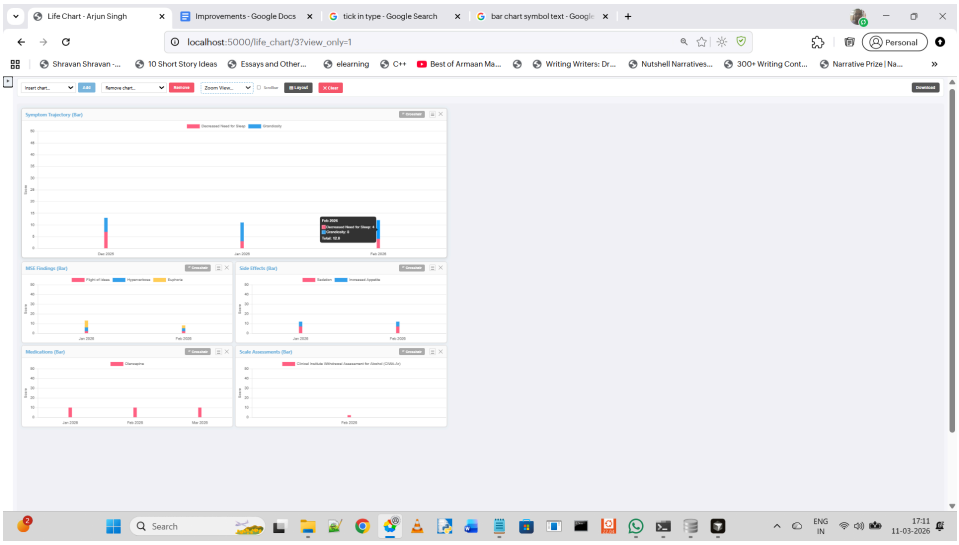
LifeChart

1. The Line Chart Remains as it is.

2. Easy zoom select option dropdown, to select, Year View, Month view, Week view aCustom date View.

3. Add a New Chart Option





COMPONENT BAR CHART – Month Wise only, Components are the symptoms.

i.e. Let's say they have entered 5 symptoms, so the total max score will be 50. Based on the score of the particular symptoms, the height of the component depends.

4. On the Line chart, Each Stressor is marked with a Clinical marker, I'll share the Clinical with you. That Clinical marker will be represented on the y axis on that particular date. (Same for Event markers)

    1. Familial or Relationship issues
    2. Loss, Bereavement
    3. Divorce / Separation
    4. Academic/ occupational Stress
    5. Financial/ Legal
    6. Medical/ Biological
    7. Physical or Sexual Violence
       (Right to them, a option to select it and after selection, a small column appears to specify about it or add a note about it)

    The Clinical markers along with the New Logo, has been, uploaded in the Images folder of the Lifechart drive.

5. In the Same way, even Event markers, will be marked on the line chart.

Event Markers include, (can we do an Interactive entry, Where We can enter direct by clicking on the life, i.e on Double clickiing the way, a dialogue box appears, it should be editable and there an Option to add Stressor and Event marker should be available)

    1. Hospitalization
    2. Suicide
    3. Major Medication Change
    4. Stoppage of medications
    5. Marriage
    6. Menarche/ Menopause
    7. ECT or any Intervention given

8. Major Medical event.

9. Legal issues
(Event markers will be placed, between the stressors and premorbid personality, in the Visit entry form and will be named as “MAJOR EVENTS”, and just like the other two a drop down menu will come with these options)

6. State Bands

1. Recovery

2. Remission

3. Partial Remission

4. Relapse

5. Hospitalisation

6. Loss of Follow-up

(Recovery → Soft Sage Green

Remission → Soft Blue

Partial Remission → Light Teal

Relapse → Muted Rust

Hospitalisation → Deep Slate Purple

Loss of Follow-up → Cool Grey)

(This will be placed after the Diagnosis, with the Heading, “Current Clinical State”, on selecting it a drop down menu comes to select between these options)

This State should be will be represented by a “thin horizontal clinical state strip” which is colour coded as and places in the top of the chart above the symptoms trajectory.

7. In Symptom Chart - Substance Abuse in Background.
1. The Substance abuse pattern, in the bottom of the chart, with component bar charts.

8. Adherence Pattern Strip

Within the symptoms trajectory graph, above the x axis, "thin horizontal medication adherence strip". If adherent completely to medication, a dark green shade and if not at all taking meds, a burnt amber shade and if in between a lighter shade on either side respectively. This will be with dim colour and reduced transperancy.

The same strip will be place in the top of the Medications charts, where the colours are normal and no transperancy

9. Zoom:

- X-axis zoom – i.e by hovering on the x axis and if we zoom in or out, only x axis part zooms in or out
- Y-axis zoom – Same

21/02/2026

1. Diagnosis-aware state options.

When a doctor selects a diagnosis, the “Clinical State” dropdown adapts, and the selected state renders in the state strip.

1 Bipolar Disorder (I & II)

States:

Recovery

Remission

Partial Remission

Depressive Episode

Manic Episode

Hypomanic Episode

Mixed Episode

Relapse

Loss of Follow-up

2 Major Depressive Disorder (Recurrent)

States:

Recovery

Remission

Partial Remission

Mild Depressive Episode

Moderate Depressive Episode

Severe Depressive Episode

Relapse

Loss of Follow-up

3 Schizophrenia Spectrum Disorders (i.e., Schizophrenia, Schizoaffective, or any psychotic disorder)

States:

Recovery

Remission

Partial Remission

Stable Phase

Acute Psychotic Episode

Residual Phase

Relapse

Loss of Follow-up

4 Obsessive Compulsive Disorder

States:

Recovery

Remission

Partial Remission

Active OCD

Severe OCD

Treatment Resistant Phase

Relapse

Loss of Follow-up

5 Anxiety Disorders (GAD, Panic Disorder)

States:

Recovery

Remission

Partial Remission

Active Anxiety

Acute Panic Phase (optional for panic disorder)

Relapse

Loss of Follow-up

Please provide colors for each state based on basic research. We can perfect them later after feedback.

Custom state option: When the dropdown of all states appears, there should also be an option to type in a custom state.

If there are multiple diagnoses, both states will appear in the dropdown, and the doctor will select accordingly.

2. For substance use entry:

After "Symptoms," add a separate column with the heading "Substance Use History."

Select Substance: A dropdown list of substances.

Then select pattern: A dropdown with options: 1. Occasional, 2. Weekends, 3. Daily, 4. Severe/Early Morning intake.

Here, also provide options to enter dates.

If this option is chosen, the user selects a substance, then has the option to select dates and similar multiple options.

Also, include an option to add a note.

Each substance gets its own thin horizontal track:

Dotted = occasional

Light shade = Weekends

Medium shade = Daily

Dark shade = severe/Early morning intake.

Alcohol → Muted amber

Hex direction: #B3742A (base) Lighter: #E3B07A Darker: #8C4F16

Tobacco (Smoked) → Charcoal Grey

Hex: #5E5E5E (base) Light: #A0A0A0 Dark: #3A3A3A

Smokeless Tobacco → Dusty Brown

Hex: #8B6B4A (base) Light: #C6A889 Dark: #5F472F

Cannabis → Muted Olive Green

Hex: #6C7A3C (base) Light: #A6B574 Dark: #4A5426

Opioids → Deep Slate Blue

Hex: #3E4F6A (base) Light: #7D8DA6 Dark: #2A3649

Stimulants → Muted Rust Orange

Hex: #A35A2B (base) Light: #D59B74 Dark: #703B18

Clicking on them displays the notes and complete details.

3. The doctor should be able to select each chart they need and remove those they don't. These charts should also be available to add or remove in the "select chart" option.

The stressor and the event markers will also be visible on the clinical state band and the chart. And there should be option to cancel them.

27 02 2026

1. **Medication Options After Life Chart:** Ensure that all options, beginning with "Medications" in the Performa, are also accessible immediately following the Life Chart view.[implemented]
2. **Medication Chart Display and Zoom Limits:**
    - Restrict the zoom range on the Medications chart to between -100% and +200%.
    - When hovering over or pressing a point on the chart, only the patient's current (actual) dose should be displayed. Do not show RD (Recommended Dose), percentage changes, or equivalent doses.
3. **Medication Chart Dot Differentiation:** The dots representing daily entries in the medication chart should be smaller than the dot representing the actual date the entry was made. This will allow the user to easily distinguish between the two.[implemented]

4. Upon clicking any point on the chart, a visual indicator—a central point with dashed lines extending in all four cardinal directions (North, East, South, West)—will appear. At any intersection where these dashed lines cross a graph line or strip, the corresponding data

details for that specific point will be displayed. [implemented]

05/03/2026

1. The drug adherence strip should be redesigned with the following improvements:

- **Integrated Drug Graph:** Include a small graph of drug information within the adherence strip itself.
- **Default Size:** The adherence strip should be slightly larger than other standard strips, but not as large as the isolated graph visualizations.
- **Zoom Functionality:** Implement a dedicated zoom option for this specific strip. When zoomed, it should display detailed drug information, mirroring the level of detail found in the isolated chart.

08/03/2026

1. **Adherence Chart Improvements**

The Adherence strip will feature a miniature version of the Adherence Chart, designed for enhanced visibility and interaction.

Default Presentation and Sizing:

- The Adherence strip will be **1.8 times** the height of other strips by default.
- The maximum height achievable will be **3 times** the height of other strips.
- Height expansion occurs only along the **x-axis** (visual indication of expandability should be present).
- Double-clicking the x-axis of the Adherence chart will toggle its height between the default and double-height.

Key Features and Interaction:

- **Interactive Chart:** The Adherence strip functions as a secondary interactive chart.
- **Synchronized Interactions:** All interactions (zoom, hover, double-click behavior) are synchronized with the main chart.
    - Zooming on the symptom chart will zoom the adherence chart, and vice versa.
    - Hover and double-click actions are identical to the main chart.
- **Bidirectional Propagation:** Interactions made on either the main chart or the adherence chart will propagate to the other.

2. Minimize the wasted space in the side margins as much as possible.
3. The crosshair feature will be enabled by default when moving the cursor. The X and Y axis values will be displayed near their respective positions as floating tags. The transparency of the crosshair's dotted line should be reduced to 65%.

Point Interaction:

Hovering near a data point (dot):

a. The dot slightly enlarges.
b. The cursor changes to a hand pointer.
c. The crosshair snaps to the point if the cursor is within approximately 10–15

pixels.

Clicking a data point (only the dots are currently clickable):

d. All vertical values and only the score for the clicked point horizontally will be displayed.
e. Corresponding segments in other layers will be highlighted.

4. When hovering over the chart, only the main title or primary inference should be displayed. The date and score should be removed from the hover display, as the crosshair will now show the date and score on the X and Y axes.
5. Major events and stressors should be places, on a separate small strip, just above the x axis.

16/03/2026

Requested System Improvements for Lifechart

This list outlines necessary improvements for the Lifechart feature, addressing data consistency and visualization issues:

1. **Data Inconsistency:** Follow-up visits deleted from a patient's profile are not being removed from the database or the Lifechart view.
2. **Missing Visual Indicators (New Visits):** The color strip representing substance use is not displayed for new visits.
3. **Missing Visual Indicators (Medication adherence):** The background color strip intended to represent medication adherence is not appearing.
4. **Missing Visual Indicators (Clinical States):** The clinical states color strip, located at the top of the Lifechart, is not appearing.
5. **Tooltip Display Logic:** When hovering over a data point, the tooltip should *only* display the Name or respective title of the symptom/data. The date (x-axis info) and score (y-axis info) should be excluded from the tooltip, as this information is already provided by the crosshair functionality.

19-03-2026

I. Chart Display and Interactivity

- **Dynamic Y-Axis Hover Behavior:** The current chart display fails to dynamically update the y-axis data when the cursor moves. The displayed value remains fixed to a single point, even as the cursor moves along or away from it. To improve interactivity, a horizontal dotted line with slightly less transparency than the existing vertical line should appear alongside the vertical dotted line, showing corresponding details. The vertical line should maintain greater visibility as it is used more frequently.
- **Dark Mode Sub-chart Visibility:** When using Dark Mode, the background of the Medications sub-chart remains light, which impairs the visibility of embedded graphs, sometimes rendering them invisible.

II. Data Representation and Color Coding

- **Distinguishing Tobacco Types:** The current color scheme needs to be refined to provide distinct color coding for "Tobacco" and "Non-Smoked Tobacco." The new scheme must accurately reflect the unique pattern of use for each.

PERFORMA

27 02 2026

1. Order – Premorbid Personality traits, Major Events, Stressors, then Adverse childhood Experiences [implemented]

(Change Premorbid personality to Premorbid personality traits)

2. In Adverse childhood experiences (Drop down) [implemented]

    a. Abuse (On selection, it expands)

        i. Emotional Abuse

        ii. Physical Abuse

        iii. Sexual Abuse

    b. Neglect (On selection, it expands)

        i. Emotional neglect

        ii. Physical Neglect

    c. Household Dysfunction

        i. Parental substance use

        ii. Parental mental illness

        iii. Domestic violence exposure

        iv. Parental separation/divorce

    d. Loss/Attachment Disruption

        i. Early parental loss (death)

        ii. Prolonged separation from caregiver

    e. Peer & Environmental Trauma

        i. Bullying (Chronic)

        ii. Severe academic humiliation

        iii. Community violence exposure

iv. Natural Disaster exposure

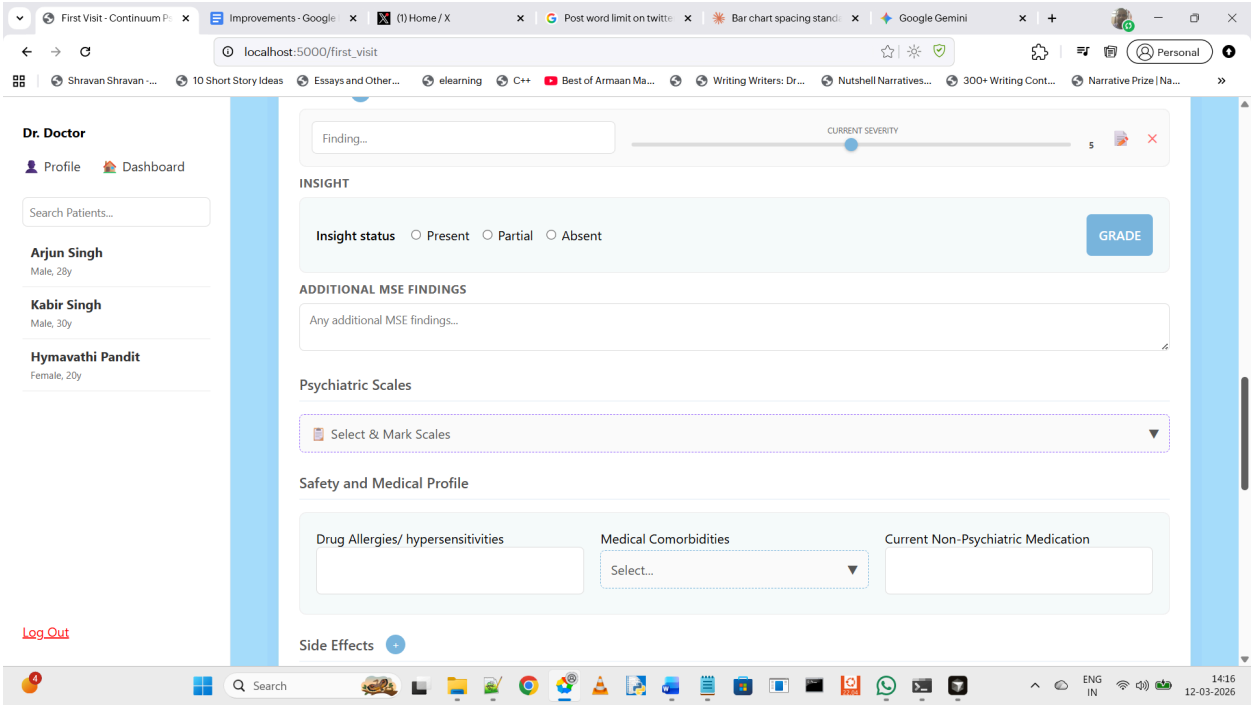
v. War/conflict exposure

f. When any Adverse childhood Experiences is enter, An option to enter “the Age of Exposure” in years.

g. Trauma Burden option – Mild/ Moderate / Severe.

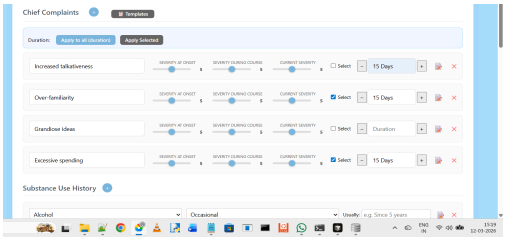
3. For all of these, Premorbid Personality, Major Events, Stressors – Remove the option of date range. Give an option to select a single date, then an option “Select range” (Optional). [implemented]

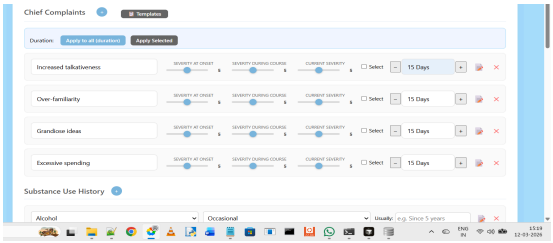
4. Place SCALES after MSE.



5. When we select the chief complaints through templates. After selecting the duration for first symptom, an option appears “Apply to all (duration)” and “Apply Selected” and an option to select appears in the duration place of all the other symptoms, can select the needed and click on “Apply Selected”, because usually most symptoms start together. And next to the Duration mention tab, “+” & “-“options, to increase and decrease the selected days or weeks

(Whatever is selected)





6. Let Provisional or Differential Diagnosis tab, be not resizable. Give a Standard a bit bigger space for them. [implemented]

7. While typing the diagnosis, a drop down of common diagnosis (like suggestion in google search or mail come) can select them or type. [implemented]

8. In current clinic state, Not need to again select the diagnosis, based on the diagnosis entered in the Provisional diagnosis, the options should be changed and displayed.

9. After adding the Next Follow date, another option will be there to add the “Type of Next Follow up” with the drop down, of the ones listed below

   i. New Registration
   ii. Extended Follow up
   iii. Follow up
   iv. Therapy Session
   v. Review/ Report Discussion

10. Premorbid Personality traits.[Implemented]

The following categories represent different facets of personality and functioning, often used in assessment or description:

Temperament (Multiple Select Dropdown)

- Introverted
- Extroverted
- Emotionally reactive
- Irritable
- Anxious
- Cyclothymic traits
- Hyperthymic traits

Social Style (Multiple Select Dropdown)

- Socially withdrawn
- Few close relationships
- Dependent on others
- Interpersonally sensitive
- Conflict-prone

Work / Functioning Pattern (Multiple Select Dropdown)

- Highly driven / Overachiever
- Perfectionistic
- Procrastinating
- Poor initiative
- Rigid / black-and-white thinking

Emotional Regulation Style (Multiple Select Dropdown)

- Avoidant coping
- Ruminative
- Emotionally expressive
- Emotionally inhibited
- Difficulty handling criticism
- Poor stress tolerance

Impulse / Behavioral Regulation (Multiple Select Dropdown)

- Impulsive
- Risk-taking
- Sensation-seeking
- Poor frustration tolerance
- Aggressive tendencies

Cluster A-like Traits:

1. Suspicious / mistrustful style
2. Social withdrawal / detachment
3. Few close relationships
4. Odd or eccentric thinking style

Cluster B-like Traits:

1. Impulsivity
2. Emotional instability
3. Poor frustration tolerance
4. Aggressive tendencies
5. Attention-seeking / dramatic interpersonal style
6. Risk-taking behavior

Cluster C-like Traits:

1. Excessive dependency in relationships
2. Avoidant interpersonal style
3. Perfectionism / overcontrol
4. Interpersonal hypersensitivity
5. High anxiety-proneness

OCEAN Model Traits (Big Five Personality)

- Openness
- Conscientiousness
- Extraversion
- Agreeableness
- Neuroticism

1. In Major Events, Add [Implemented]
    a. Significant Migration/ Relocation
    b. Suicide Attempt
    c. Suicide in family
2. In MSE, after thought, perception and Affect, [implemented]
    a. 4th option - Insight - 3 options - Present, Partial, Absent. After that an option "GRADE" on clicking on it, Grades of insight dialogue box, pop ups. Grade 1 to 6
        i. Complete Denial of illness
        ii. Slight Awareness of being sick but denying at the same time
        iii. Awareness of being sick but blaming on external/ organic factors
        iv. Awareness that illness is caused bt something unknown in the pt
        v. Intellectual insight
        vi. True emotional insight
    b. 5th option - Additional MSE Findings - With an empty Text box (Note Box)
3. In Medical Co-morbidities. A drop down. (Multi-select) [implemented]
    a. Diabetes mellitus
    b. Hypertension

    c. Hypothyroidism
    d. Hyperthyroidism
    e. Epilepsy
    f. COPD
    g. Asthma
    h. Tuberculosis
    i. HIV
    j. OTHERS, an Option to Specify
4. Family History of Psychiatric illness (After Substance use history, before MSE) [implemented]
    a. Present/Absent, If present dropdown expands to multi select.
        i. Depression
        ii. Bipolar disorder
        iii. Schizophrenia/Psychotic disorder
        iv. Anxiety disorder
        v. Substance use disorder
        vi. Suicide
        vii. Personality Disorder
        viii. Neurodevelopmental disorder
        ix. Neurological disorder
        x. Unknown psychiatric illness
        xi. OTHERS - Specify (Free Text)
5. Equivalent Doses - after the calculation, we should round off it to the nearest available dose. (I'll send the list of available doses in excel)
6. During Tapering, lets do it as it is in the video. I.e when "taper" is selected, the drug name should not be repeated. Instead, a line or arrow should extend down from the preceding column, and the subsequent columns (starting from the dose) should then appear.
7. In the Follow-up visit context, an option to mark adherence for each prescribed drug should be displayed, as the patient's previous medication list is visible. This adherence marking will automatically calculate and display the patient's overall status (Adherent, Non-Adherent, or Partially Adherent) in the "Strip of Life chart". Additionally, if a specific drug is marked as Non-Adherent or Partially Adherent, its corresponding graph line on the chart should appear as a dotted line.

05/03/2026

1. History of Developmental Milestone Delay (After family history, before MSE) [implemented]

    Options:

- No delay reported
- Delay reported
- Unknown

If Delay reported:

Type of Delay (multi-select)

- Speech / language delay
- Motor delay
- Social developmental delay
- Learning delay
- Global developmental delay
- Not sure

Optional: Notes

2. For both new registrations and follow-up entries, users should have the option to modify the date. This is necessary because users may need to input historical data into the platform to obtain accurate inferences.
3. Substance abuse history

Detailed Substance Use History Intake Form

This section is designed to gather a comprehensive history of substance use for each substance reported. Please complete the following details for every substance an individual has used.Substance-Specific Information

For each substance reported, please provide the following:1. Substance Identification

- **Substance:** Please specify the type of substance used (e.g., Alcohol, Cannabis/Marijuana, Tobacco/Nicotine Products, Cocaine, Heroin, Prescription Opioids, Sedatives, Stimulants, Hallucinogens, Inhalants, Methamphetamine, etc.).

2. Initiation and Duration of Use

- **Age at First Use or Approximate Start Year:** $\underline{\hspace{2cm}}$ years.

    - *(Note: This age/year will serve as a critical reference point for visual representation on longitudinal graphs and timelines.)*
- **Duration of Use (Total):** $\underline{\hspace{2cm}}$ years/months (Estimate the total period during which the substance was used, regardless of current status).

3. Pattern and Intensity of Use

- **Pattern of Use:** Please select the description that best reflects the typical or most prominent pattern of use during the period of active use.
    - **Occasional/Experimental:** Infrequent, limited use (e.g., once a month or less, trying it out).
    - **Weekends/Social:** Use primarily confined to weekends, social gatherings, or holidays.
    - **Daily/Regular:** Use occurring almost every day or multiple times per week.
    - **Severe/Heavy Daily Use:** High-frequency, high-volume use; often including early morning intake (e.g., "eye-opener" use to manage withdrawal or maintain function).
    - **Binge:** Use of a large amount of the substance over a short period with the intention of becoming intoxicated (e.g., heavy episodic drinking).
- **Peak Frequency/Quantity:** Please describe the highest level of use experienced (e.g., "10 drinks per day," "1 gram of cocaine per week," "2 packs of cigarettes per day").
    - Frequency: $\underline{\hspace{5cm}}$
    - Quantity: $\underline{\hspace{5cm}}$

4. Current Status of Use

- **Current Status:** Please indicate the individual's relationship with the substance as of today.
    - **Ongoing Use:** The substance is currently being used (even if infrequently).
    - **Currently Abstinent:** The individual has completely stopped using the substance.
    - **Past Use:** Use has stopped, but the individual does not identify as currently abstinent (e.g., stopped many years ago, or use was minimal/experimental).

-----Details for Ongoing Use or AbstinenceIf Currently Abstinent:

- **Abstinent Since:** Please specify the precise date or approximate time frame when the last use occurred.
    - Month: $\underline{\hspace{2cm}}$ / Year: $\underline{\hspace{2cm}}$
    - *(This information is crucial for calculating the current length of sobriety.)*

If Ongoing Use:

- **Current Frequency/Quantity:** Describe the individual's current pattern of use for this substance.
    - Frequency: $\underline{\hspace{5cm}}$

    - Quantity: $\underline{\hspace{5cm}}$
- **History of Abstinence:** Has the individual ever attempted or achieved a period of complete abstinence from this substance since first use?
    - Yes
    - No

If History of Abstinence is Yes:

- **Longest Abstinence Period:** Please report the single longest period the individual went without using this substance.
    - $\underline{\hspace{2cm}}$ months / $\underline{\hspace{2cm}}$ years
    - **Circumstances Surrounding Longest Abstinence:** Briefly note what facilitated or motivated this period of abstinence (e.g., treatment, pregnancy, health scare, legal issues). $\underline{\hspace{10cm}}$

-----Additional Context and Clinical Observations9. Notes:

- **Relevant Context:** Please use this space to record any critical contextual information, such as:
    - Method of administration (e.g., smoked, ingested, IV use).
    - History of tolerance or withdrawal symptoms.
    - Consequences experienced due to use (e.g., legal, occupational, health, relational).
    - Treatment history for this substance (e.g., detox, rehab, therapy, self-help groups).
    - Any other observations or details relevant to the individual's history with this substance.

$\underline{\hspace{15cm}}$
$\underline{\hspace{15cm}}$

$\underline{\hspace{15cm}}$

4. **Diagnosis System Expansion**

Enhance the system to allow for the addition of multiple diagnoses. The available options for diagnosis selection/type need to be expanded to encompass all classifications from ICD-10, ICD-11, and DSM-5. Furthermore, the ability to view the diagnostic criteria should also be extended to include ICD-11.

5. The same type/select functionality, along with the ability to add multiple entries, should be applied to Differentials as well, mirroring the existing capability.

6. Regarding Tapering, we should align the functionality exactly with the reference video. When "taper" is selected, an arrow should extend downwards from the drug name (the drug name should not be repeated). All information following the drug name should then be displayed below after the arrow. Try to make it, as it is in the video
Effortless Medicine Tapering with TatvaPractice

7. **Proposed Improvements for Patient Identification/Recall Field**

1. **Rename Field:** Change "Identification Notes (Doctor only)" to "Patient Recall Cue" (abbreviated as "RC" on the appointment screen).
2. **Enhance User Interface:**
    - **Private Note:** Place small grey text below the field: "*Private note visible only to the doctor.*" This maintains a **clean yet informative** interface.
    - **Placeholder Text:** Include a **placeholder text** within the field to guide users and improve data quality:
      "Short phrase to help you recognize the patient
      (e.g., Teacher with bipolar, Tall man with tremor)"

8. Previous Psychiatric Treatment

○ No previous treatment

○ Previously treated but not on treatment now

○ Currently on treatment

If 2-3,

- **Previous Diagnosis**

[dropdown multi-select]

- **Medication History**

[text or medication list]

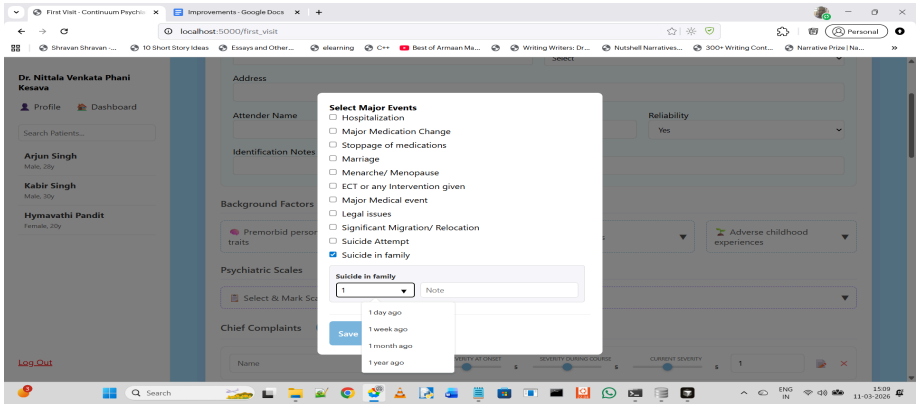
- **Duration of illness.**

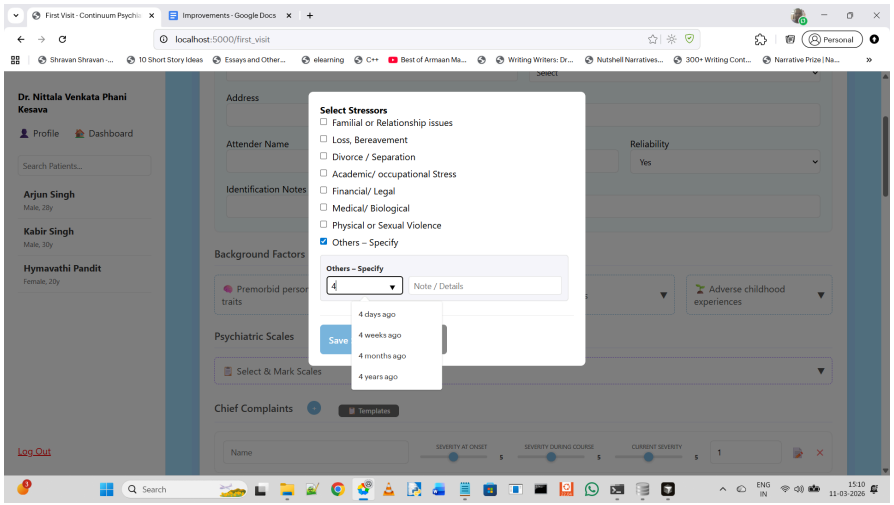
11/03/2026

Suggested UI/UX Improvements

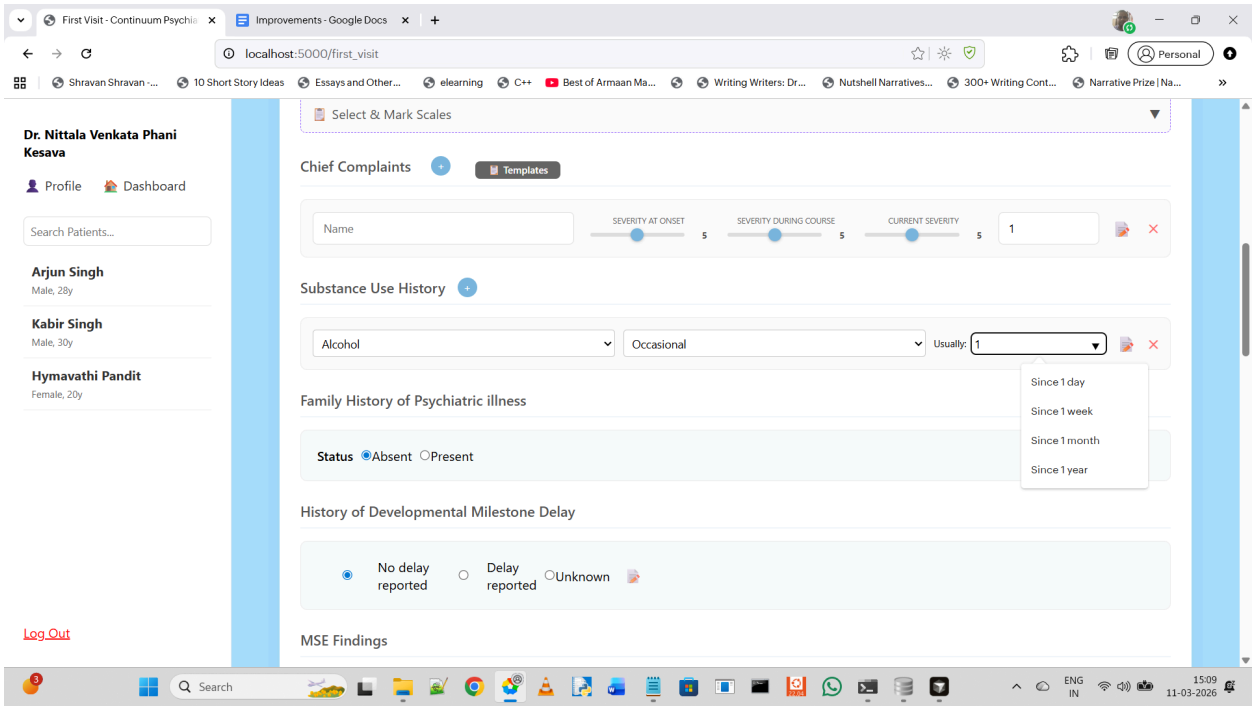
Date/Duration Input and Display:

1. **Major Events/Stressors:** When entering duration only, the system should calculate and display the time using "ago" in a dropdown (e.g., "5 days ago," "5 months ago").





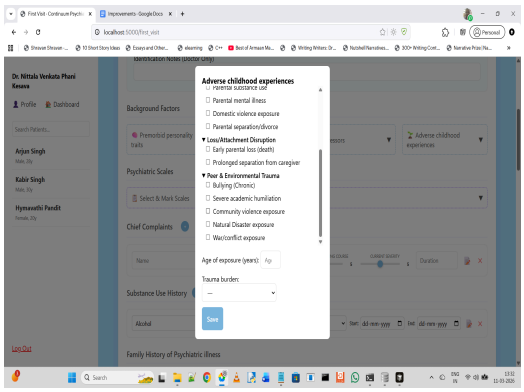
2. **Substance Use:** Remove date fields. Implement a "usually" dropdown with "Since" as the prefix (e.g., "Since 5 days," "Since 5 months," "Since 5 years").

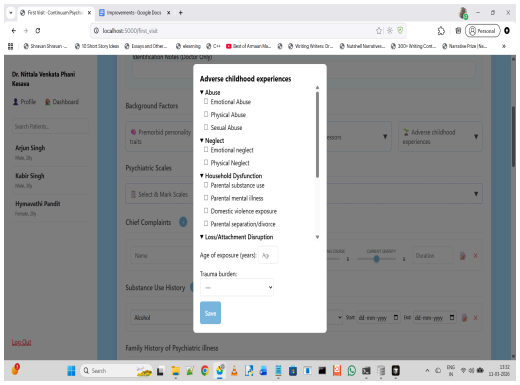


3. **Singular/Plural Consistency:** Correct the display to use singular forms when appropriate (e.g., "1 month" instead of "1 months," "1 day" instead of "1 days").

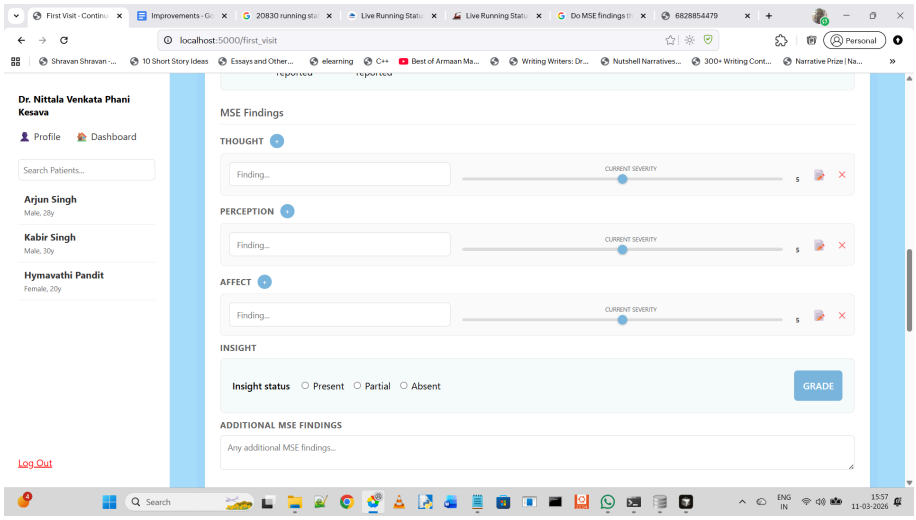
Screen Navigation and Layout:

1. **Adverse Childhood Experiences (ACE) Screen:** Ensure the screen is fully scrollable to allow users to reach and select the "save" option. (Reference image attached.) [Implemented]



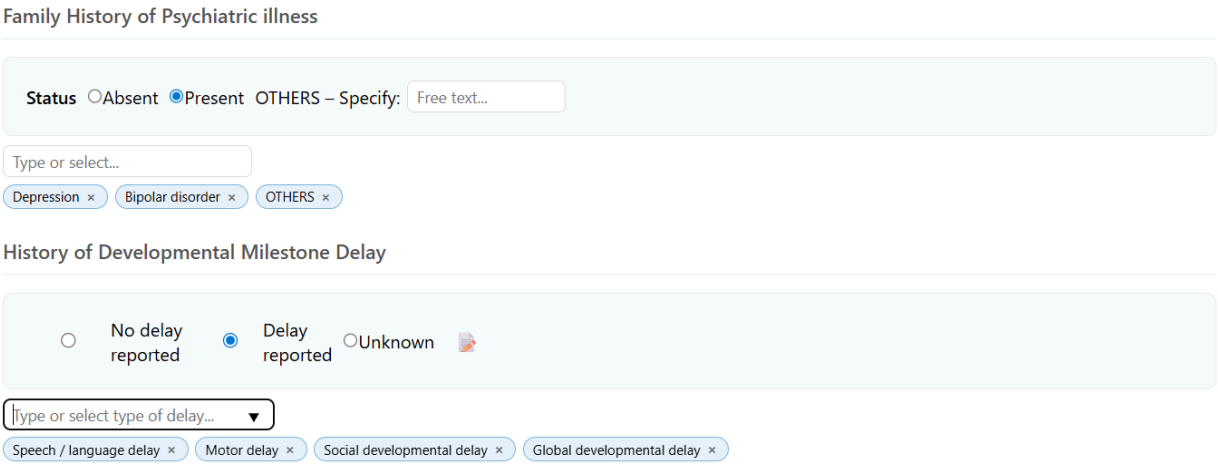


2. **MSE (Mental State Examination):** Simplify the display in the MSE section. Only the "current severity" bar is needed; remove the other two bars and the duration field, as this is a cross-sectional examination. The "note" option should remain.

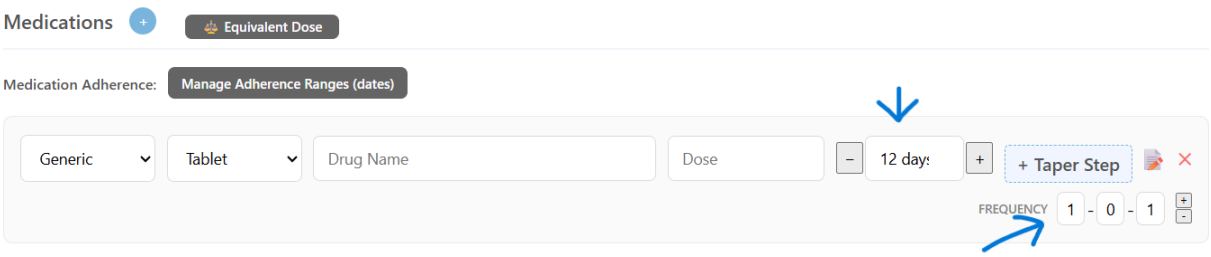


Search and Input Functionality:

1. **Family History:** Implement a dropdown for common entries. Allow free text search assisted by the dropdown suggestions. [Implemented]



2. **Medication Dosage:** Add non-text input for adjusting dosage. Provide "+" and "-" buttons next to the dose field to increase or decrease the amount (similar to the frequency adjustment feature). [Implemented]



3. **Frequency Input Glitch:** Prevent the appearance of an extra input box when the space bar is pressed in the third frequency field. The new box should only appear when the "+"

or "-" buttons are clicked. [Implemented]

Information Display and Workflow:

1. **Historical Data Display:** In subsequent follow-up visits (or during the first visit for certain sections), the data entered in the last visit for sections like Stressors, Premorbid Personality, Major Events, Childhood Events, Comorbid Medical Condition, etc., should be displayed. This historical data should appear on a small badge on the performance review, enclosed in box brackets, using a smaller and lighter font.
2. **Follow-up Type:** The "Type of follow-up" field should be available immediately after the initial registration is complete.

16-03-2026

1. In follow up visit, The Substance Name, once entered, should not be allowed to change or delete. The fixed variable should not be allowed to change, but only option to select the current status, pattern of use and duration (not to select year or month, but in blank space, how in other places we enter duration, but with a prefix since) should be available, and that when interpreted on the life chart, the exact date from entered days to present days will be calculated and represented according. Because we know till the last visit what happened, so we are asking for what happened in the last 30 days only.
2. We should completely remove the “past use” option from follow up tab.
3. And here, if the doctor selects “Currently abstinent”, “pattern of use” should not come, only duration (since) should be displayed.
4. But, if the doctors add a new substance in the followup visit, which was not there before. Then it should exactly how it is now, as in the first visit performa.**Proposed**

16-03-2026

To improve the first entry chart for family history documentation:

1. When a family history is present, an option to select the degree of the relative should be provided alongside the diagnosis.
2. The degree of relative selection should be a drop-down menu with the following options:
    - First Degree relative
    - Second degree relative
    - Third degree relative

16-03-2026

Improvements for Follow-up Visits:

1)SUBSTANCE USE

1. **Substance Information Locking:** Once the Substance Name and the age of onset is entered during an initial visit, it should be made immutable (cannot be changed or deleted) in subsequent follow-up visits. But it should be available for display (Read only) in follow up, visit, just for reference.
2. **Fixed Variables & Duration:** For the selected substance, fixed variables should not be editable. Only the current status, pattern of use, and duration can be updated.
    - **Duration Input:** The duration should be entered in a blank space, similar to other duration fields, with a "since" prefix (e.g., "since 3 days ago"), instead of selecting months or years.
    - **Life Chart Interpretation:** The system should calculate the exact date range from the entered "days since" value to the present and represent this on the life chart. This calculation should specifically focus on the changes observed since the last visit (e.g., in the last 30 days).
3. **Removal of "Past Use" Option:** The "past use" option should be completely removed from the follow-up tab.
4. **Handling "Currently Abstinent":** If the clinician selects "Currently abstinent," the "pattern of use" field should be suppressed, and only the "duration (since)" field should be displayed.
5. **Adding a New Substance:** If a new substance (not recorded previously) is added during a follow-up visit, the input process should exactly mirror the current procedure used in the initial visit performa.

In the Main Performa or if added new in followup.

If the use selects Past use in pattern of use, then the pattern will be ask, then “Total Duration of Use” option should be given, they will enter number and drop down of months, year comes. And then the option of “Abstinent since” should be asked, there again an option to add number, and it will show “Since - months, years, option”

2)Suggested Improvements for 'Pattern of Use' Data Entry

These improvements apply to both the Main Performa and new entries in the followup section.

Scenario: User selects 'Past use' in the 'Pattern of Use' field.

1. **"Total Duration of Use"**: A field should be presented where the user can enter a number, followed by a dropdown menu offering "months" or "years".
2. **"Abstinent since"**: Following the "Total Duration of Use," this option should be presented. It should allow the user to input a number and select the appropriate time unit from a dropdown menu ("months" or "years") with a prefix of “Since” to indicate the duration of abstinence.

3)Prefix & Suffix

These Prefix and suffix, matter a lot to calculator the exact dates and there by represent that in the life chart. We should make sure it is calculating the dates with the entry date as reference and is represented in life chart according.Accurate calculation of dates in the life chart depends significantly on correctly handling prefixes and suffixes. Therefore, it is crucial to ensure that the date calculation references the entry date precisely and that the resulting data is accurately represented in the life chart.

4)FAMILY HISTORY

For the Family History field, the option to change the default value should not be available. The previously entered value should be displayed in a read-only format. A small "pencil" icon should be provided to allow editing or updating.

Only when the user clicks the edit icon should the option to change the history appear in a separate dialogue box. This dialogue should include a caution: "Changing these values will erase the previously entered information" or "Changes will be applied to all previous and future dates." Once updated, the change should be reflected in all instances, both past and future.

Display Examples:

- Family history: Present - Schizophrenia - second degree relative Or
- Family history - Absent

Then under the same family History Heading.
Consanguinity - Yes/no/unknown.

5) Developmental Milestone Delay

Follow the same procedure as for the "Family history" section, but omit the "Degree of relative" field. All other elements should remain identical.

6) Medication Adherence Management

The section currently labeled "Manage Adherence ranges" should be renamed to **"Medication Adherence."**

Input Field Modifications:

- Users must be restricted to selecting **only one** adherence option.
- If either "Partial Adherence" or "No Adherence" is selected, the input field for the date range should be replaced. Instead, the user will enter a number with a corresponding dropdown (e.g., days, weeks, months) and the prefix "Since."

Chart Representation Logic:

- The start date for charting adherence will be calculated using this new "Since" entry.
- Adherence data from the *previous* visit will be carried forward and displayed on the chart up until the start date calculated from the new "Since" entry.

7) Clinical States

For Clinical States, the system should be configured to allow only a single entry.

19-03-2026

1. Insert a new tab labeled "**Functional Impairment**" immediately following the "Negative symptoms" section. This tab should contain the following 5 questions, each using a slider for a rating from 1 to 10.
    a. How affected is the patient's work or academic functioning?
    b. How affected are the patient's social interactions?
    c. How affected are the patient's relationships and responsibilities at home?
    d. How affected is the patient's ability to take care of personal needs?
    e. How affected is the patient's engagement in leisure or enjoyable activities?
2. Insert a new tab of Negative history, after the "Symptoms", with options of Yes, No & unknown for all. With unknown as default for all. (This will only be present in the first registration, in follow up, only the positive findings should be displayed in a read-only format. A small "pencil" icon should be provided to allow editing or updating). If they select Yes, the options expand accordingly, as mentioned below. (Severity, Duration (with 'ago' suffix), and sequelae).
    a. H/o Head injury
    b. H/o Seizures
    c. H/o fever with Altered Sensorium
    d. H/o focal neurological deficits/headache
    e. H/o Hallucinatory behaviour
    f. H/o Delusions or suspicious
    g. H/o Elated mood/ decreased need for sleep
    h. H/o Persistant Low Mood.
    i. H/o Repetitive thoughts or Behaviours
    j. H/o Suicidal attempts

k. H/o Legal Issues

If Age above 60,

l. H/o progressive memory impairment
m. H/o decline in daily functioning

If Alcohol the main complaint. (in substance abuse, they select Alcohol, these should add up in the Negative History.

1) H/o hematemesis/ melena
2) H/o jaundice
3) H/o Confusion/ Ataxia/ Memory Loss
4) H/o Ascitis/ Pancreatitis

MSE -

1. Consciousness - Clear/ drowsy/ Fluctuating/ Stupor/ Coma (All of these will be present in a line, as badges, user will select an option, then that option should be highlighted, and the highlighted one in bold will be selected on default)
2. General Appearance & Behaviour
    a. Normal/ Over dressed/ Unkempt
    b. Co-operative / Not co-operative.

If Non-Co-operative, The whole MSE will Stop here, Except Affect

Is Co-operative,

c. Rapport
    a. Established/ Not Establised / established with difficulty

If Rapport, Not established, The Whole MSE will stop here, Except Affect.

If Rapport, Established/ established with difficulty, It Continues.

a. Eye Contact : Achieved/ Not Achieved Sustained/ Not Sustained
b. Psychomotor Activity: Normal/ Decreased/ Increased.
c. Involuntary movements (with search and drop down option)
    i. Tremors
    ii. Acute Dystonia
    iii. Akathisia
    iv. Bradykinesia
    v. Tardive dyskinesia

vi. Chorea/ choreoathetoid movements
vii. Myoclonus
viii. Stereotypies
ix. Tics

3. SPEECH

a. Reaction time - Normal/ Decreased/ Increased
b. Relevance - Relavant/ Irrelevant
c. Coherence - Coherent/ Incoherent
d. Intensity - Audible/ Excessively Loud/ Soft
e. Pitch - Monotonous / Normal fluctuation
f. Ease of speech - Spontaneous/ hesitant/ Mute/ Normal

4. Thought (as it is now) but with search and drop down. List is in the drop Down Doc attached

5. Perception (as it is now) but with search and drop down. List is in the drop Down Doc attached

6. Affect (as it is now) but with search and drop down. List is in the drop Down Doc attached

7. Mood - same as above three.

8. Insight - as it is now.

SMART PRIORITIZATION LAYER FOR SEARCH DROPDOWN

The search dropdown will feature a smart prioritization layer to display relevant symptoms as the user types.

Dropdown Structure:

The dropdown will consistently show a total of **6 options** each time:

1. **Top 4 Prioritized Symptoms:** These will be the most commonly used options,

determined by a calculated **weighted score**.

2. **Last 2 Related Symptoms:** These will be similar symptoms purely based on the text being typed and will *not* consider the weighted score.

Prioritization Mechanism (Weighted Score):

The weighted score, used only for the top 4 slots, is calculated to personalize the order based on a user's past usage. This scoring should be updated once a day and is specific to the individual user profile, not global.

Score Formula:

$$ \text{Score} = \sum (\text{w}_\text{d}) + \text{B}_\text{r} $$

Where:

- $ \sum (\text{w}_\text{d}) $ = **Decayed Frequency** (Sum of all past uses, weighted by recency).
- $ \text{B}_\text{r} $ = **Recent Bonus** (Extra boost for very recent use).

1. Decayed Frequency (w_d):

A weight is assigned to each usage based on how many days ago (d) it occurred:

$$ \text{w}_\text{d} = \frac{1}{(1 + d)} $$

Usage	Days ago (d)	Weight (w_d)
Today	0	1.00
1 day ago	1	0.50
2 days ago	2	0.33
5 days ago	5	0.16
10 days ago	10	0.09
30 days ago	30	0.03

2. Recent Bonus (B_r):

This provides a temporary boost for the most recent activities:

Recency	Bonus (B_r)
Used today	+1.0
Used in last 2 days	+0.7
Used in last 3–5 days	+0.4
Used in last 6–10 days	+0.2
Used $>$10 days ago	+0

Key Constraints:

- The same option must never appear more than once in the list of 6.
- The personalization and scoring must happen entirely in the background; the user should not be informed that the first 4 are "recommended" or differentiated from the last 2.
- All prioritization is strictly **personalized to the user's profile**.

Affect

In affect after the dropdown, the below mentioned will be listed as badges. (As we did for other MSE)

Reactivity - Present/ Absent

Range - Normal/ Restricted/ labile

Congruent/ Incongruent

Appropriate/ Inappropriate

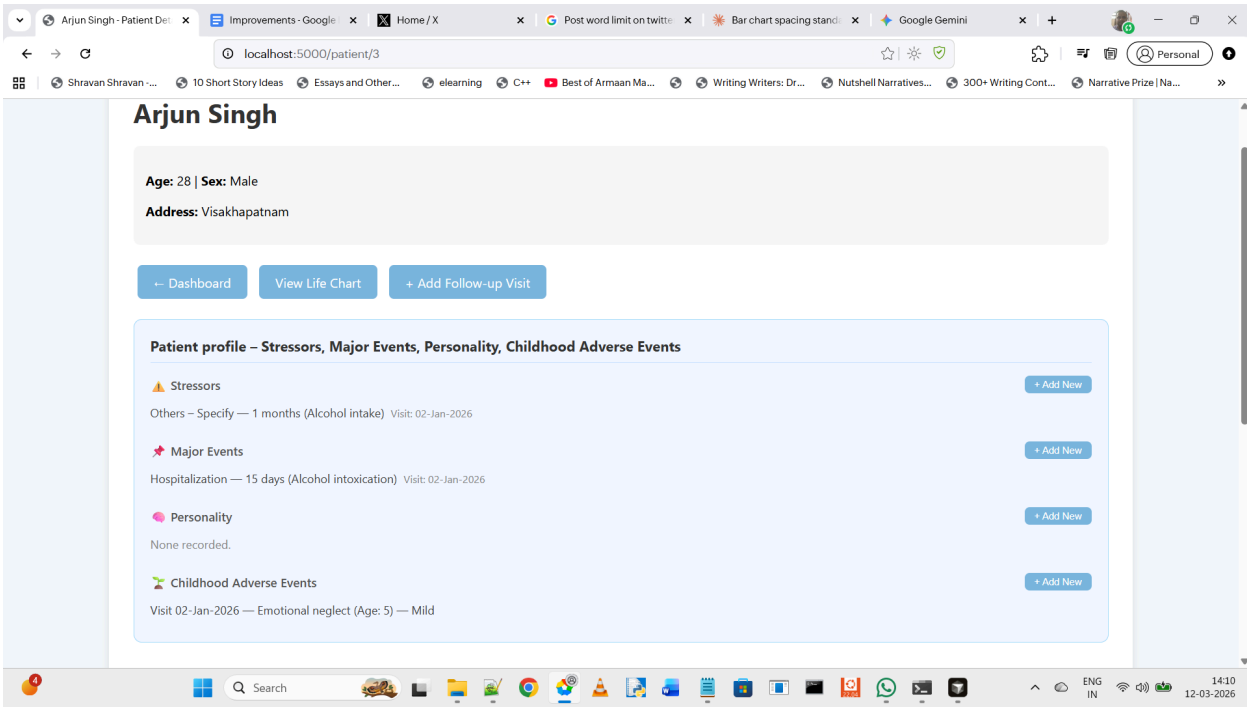
Follow up PERFORMA

11/03/2026

FOLLOW UP VISIT

Requested Improvements to System Functionality

1. **Stressor and Major Events Display and Entry:**
    - The "Stressor," "Major Events," "Personality," and "Childhood Adverse Events" entered previously should be prominently displayed on the user's badge/profile.
    - There must be an option to add new entries for these categories.
    - While the existing stressors and major events will be displayed on the badge, the internal entry fields should be cleared to allow for new data input.



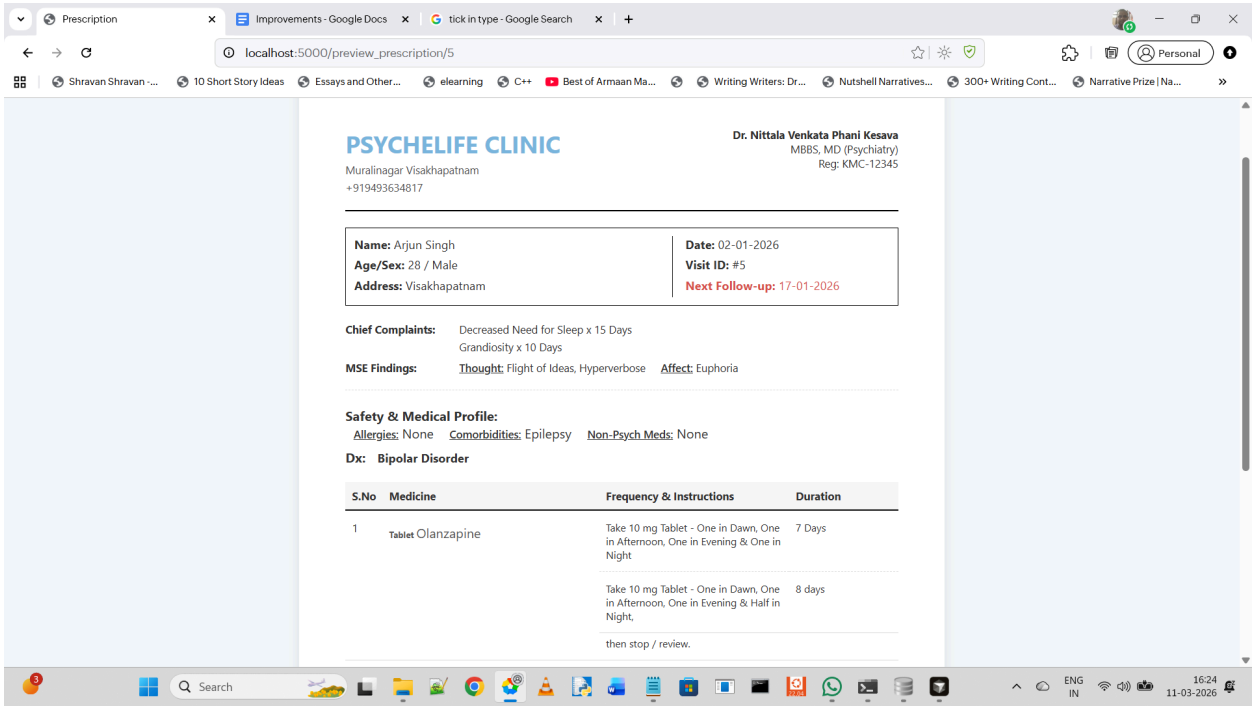
2. **Symptom Duration in Follow-up Visits:**✔
    - For symptoms carried over from a previous visit in a "Follow-up Visit," the system should **not** ask for a duration.
    - Duration should only be requested for any **new** symptoms added during the current follow-up visit.
3. **Duration Field in MSE:**✔
    - The duration field is not required in the Mental State Examination (MSE) section.

Prescription

11/03/2026

Proposed Improvements for Clinical Documentation

1. **Chief Complaints Ordering:** Present the chief complaints vertically, one beneath the other.
2. **Chronological Sequencing:** List the chief complaints in chronological order, with the symptom of the longest duration appearing first.



PSYCHELIFE CLINIC
Muralinagar Visakhapatnam
+919493634817

Dr. Nittala Venkata Phani Kesava
MBBS, MD (Psychiatry)
Reg: KMC-12345

Name: Arjun Singh
Age/Sex: 28 / Male
Address: Visakhapatnam

Date: 02-01-2026
Visit ID: #5
Next Follow-up: 17-01-2026

Chief Complaints: Decreased Need for Sleep x 15 Days
Grandiosity x 10 Days

MSE Findings: Thought: Flight of Ideas, Hyperverbose Affect: Euphoria

Safety & Medical Profile:
Allergies: None Comorbidities: Epilepsy Non-Psych Meds: None

Dx: **Bipolar Disorder**

S.No	Medicine	Frequency & Instructions	Duration
1	Tablet Olanzapine	Take 10 mg Tablet - One in Dawn, One in Afternoon, One in Evening & One in Night	7 Days
		Take 10 mg Tablet - One in Dawn, One in Afternoon, One in Evening & Half in Night,	8 days
		then stop / review.	

3. **Follow-up Prescription Logic:** When generating a prescription during a follow-up visit, calculate the duration of any carried-over symptoms from the date of the previous entry.

Prescription | Improvements - Google Docs | tick in type - Google Search

localhost:5000/preview_prescription/7

← Back to First Visit

Print / Save as PDF

PSYCHELIFE CLINIC

Muralinagar Visakhapatnam
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Dr. Nittala Venkata Phani Kesava
MBBS, MD (Psychiatry)
Reg: KMC-12345

Name: Arjun Singh	**Date:** 17-01-2026
Age/Sex: 28 / Male	**Visit ID:** #7
Address: Visakhapatnam	**Next Follow-up:** 01-02-2026

Chief Complaints: Decreased Need for Sleep x 2 weeks
Grandiosity x 2 weeks

Side Effects: Sedation x 10 Days, Increased Appetite x 10 Days

MSE Findings: Thought: Flight of Ideas, Hyperverbose Affect: Euphoria

Dx: Bipolar disorder

S.No	Medicine	Frequency & Instructions	Duration
1	[Generic] Olanzapine (Tablet)	Take 10.0 mg Tablet - One in Morning & One in Night	15 Days

Search

ENG IN 16:24 11-03-2026

Scales

1. Add WHODAS 2.0 self administered scale to the scales.
2.